

Description

Syndromic surveillance is public health surveillance that emphasizes the use of near "real-time" health data and statistical tools for disease or hazardous event detection, situation awareness for mass gatherings and public health emergencies, and ad hoc and population health trend analyses. For syndromic surveillance purposes, required data on all clinical encounters must be sent to the jurisdictional public health agency within 24 hours of the any given encounter, and in accordance with local or state laws and practices.

The IP Visit-Cardiac Surgery Test Case provides an example of an inpatient surgical visit for a patient with multiple diagnoses. In this scenario, syndromic surveillance data are sent to a state public health agency in accordance with guidelines recommended by the International Society for Disease Surveillance (i.e., once following admission, once following an update, and once following a discharge).

In this test case, a middle aged man is admitted for a pre-scheduled surgical removal of a coronary artery obstruction and insertion of stents, spends one night in the hospital for observation, and is discharged home.

This test case involves 3 steps:

- an admission message (ADT^A01),
- an update message (ADT^A08)
- a discharge message (ADT^A03).

Messages in this test case will be used in a Manual Test Step under functional requirement testing for validating the HIT capability of populating Visit ID (PV1-19.1) and Visit Time (PV1-44.1) each with the same value for all messages included in this Test Case.

Comments

No Comments

Pre-condition

No PreCondition

Post-Condition

No PostCondition

Test Objectives

This test case examines a Health IT Module's ability to create an ADT^A04-Registration message, and an ADT^A03-Discharge message within HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019, conformance requirements.

The following test objectives and variants are supported by this test case:

- Setting is inpatient.
- Chief complaint is captured as narrative in PV2-3 Admit Reason
- **Patient name is de-identified in the submissions:** Per ASTP/ONC, for certification testing HIT developers must demonstrate that their system supports the Patient Name and de-identified data in PID-5 (this policy is conformant to the named standard HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019).The patient name presented in a pseudonymized manner is required for this Test Step.
- **Diagnoses are submitted test using either ICD-10-CM or SNOMED-CT:** Per ASTP/ONC, for certification testing a Health IT Module can use either the ICD-10-CM or SNOMED CT® value sets in the submitted messages for all of the test steps in the Syndromic Surveillance Test Suite. Where the tool does not have test data that supports the Health IT Module's value set (either ICD-10-CM or SNOMED CT®), the developer of the Health IT Module must provide the codes and testers must perform a visual inspection of the messages for these test steps to ensure that equivalent and valid ICD-10-CM or SNOMED CT® are used to populate the messages. This policy is conformant to the named standard HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019). ICD-10-CM codes are provided/required in the Test Data for coding the diagnoses in this Test Case.
- Updates to Diagnoses
- Discharge Disposition Expired

Evaluation Criteria

No evaluation criteria

Notes

No Note

